

George Peptide Venture: Creative Opportunity and Blind-Spot Audit

Board memorandum — 21 August 2026

Executive answer

Yes—important matters have been missed, but the missing element is not another market report, brand campaign, peptide catalogue or customer journey. It is a coherent operating thesis that converts extensive design work into one lawful, paid, measurable system. The seven audits agree on a striking diagnosis: George has unusually strong blueprints for compliance, procurement, content, customer journeys, quality and operations, yet almost no implementation evidence. The operating lane is unchosen; responsible entities and licensed actors are unnamed; registers are blank; offers and prices are undefined; local demand is untested; and no integrated profit, cash or working-capital model exists.

The board should therefore **stop treating the venture as a peptide seller in search of a funnel**. Its strongest near-term position is an **asset-light assurance, intelligence and workflow company** serving licensed institutions and, only through separately governed arrangements, independent clinical practices. SAHPRA currently warns that unregistered peptides are being unlawfully marketed through online platforms, social media, gyms and wellness centres, and states that products intended to treat, prevent or alter bodily functions must be registered before sale.[1] This makes verification, traceability, route separation and documented professional independence more than compliance overhead: they can be the product.

The recommended sequence is:

- 1 **Choose and document the route architecture**, with separate legal, brand, data, claims and conversion boundaries.
- 2 **Sell the completed research as a regulated-product procurement intelligence subscription**, attaching fixed-fee diligence as the premium service.
- 3 **Pilot a Virtual Peptide Quality Office for legitimate research institutions**, using each RFQ and lot review to build a permissioned data asset.
- 4 **Co-design a Garden Route Clinical–Pharmacy Quality Compact and access-friction ledger**, without acquiring patient traffic until professional, privacy and product gates pass.
- 5 **Implement a small fail-closed evidence/control system**, then publish selected process proof through a Trust Ledger.

This sequence can produce paid evidence within 30–90 days without inventory, consumer medicine promotion or speculative infrastructure. It also creates four mutually reinforcing moats: **verification history, institutional workflow embedding, Garden Route access data and auditable quality evidence**.

Board challenge: six meta-patterns across the audits

1. The venture has a blueprint surplus and an evidence deficit

The project has gates, registers, journeys, budgets, claims controls, supplier matrices, brand directions and a keyword workbook. These are unusually thoughtful. However, the operating rows are mostly empty, and modeled CPC, conversion, order value and ROAS are assumptions rather than demand evidence. **The next unit of value is an executed record: a paid pilot, signed responsibility matrix, approved claim, qualified counterparty, completed lot review, tested recall, observed service time or verified renewal signal.** Commissioning more high-level research would deepen the imbalance.

2. Route ambiguity is the root risk, not a minor launch detail

Clinical care, regulated finished-dose procurement, institutional research supply, cosmetics and general education are legally and operationally different businesses. They cannot safely share one generic “peptide” identity, CRM, consent, WhatsApp number, catalogue or CTA. SAHPRA’s advertising guideline states that scheduled medicines may not be advertised unless registered and that claims may not go beyond the accepted registration evidence.[3] A route decision and firewall must therefore precede audience acquisition, not follow it.

3. The project has inverted the normal sequence by designing marketing before economics

The declared budgets and media scenarios are useful controls, but there is no offer catalogue, payer map, service price, delivery capacity, partner charge, gross margin, VAT treatment, working-capital requirement, churn definition or downside runway. **The board should not authorise paid acquisition against assumed average order values.** It should authorise only model-specific willingness-to-pay tests and a 12-month driver-based cash model.

4. George’s best early product is assurance, not molecules

Across all seven audits, the strongest reusable assets are supplier research, exact-presentation comparison, regulatory status checks, documentation review, claims governance, service routing and quality controls. These can be sold as subscriptions, retainers and fixed-fee projects without taking title to medicines. The novel strategic move is to make **verification, exceptions, expiry and change history** the product—not a superficial “medical-grade” badge.

5. The durable local moat is a Garden Route friction graph

National content is easy to copy. A dated, neutral record of local provider capacity, pharmacy handoffs, laboratory turnaround, benefit authorisation, travel constraints, referral completion, lane reliability and recurring failure points is not. George should use its geography to build a

consented, minimised, time-stamped access and service-quality dataset, while re-verifying facts at the point of decision and never ranking providers or medicines by commercial conversion.

6. Compliance must become executable infrastructure and public proof

The control workbook is only a template until named owners, approvals, evidence, permissions and kill switches exist. POPIA restricts unsolicited electronic direct marketing unless consent or the narrow existing-customer exception applies, places the burden of proving consent on the responsible party, and requires objection handling and suppression.[4] Likewise, SAHPRA's wholesaling guidance expects documented quality systems, CAPA, controlled returns, traceability and reconciled recalls.[5] A low-code control tower, claims ledger, consent ledger and Trust Ledger can turn these obligations into longitudinal evidence that competitors cannot quickly imitate.

What is already strong

The venture should preserve—not recreate—the following assets:

- **Route-aware strategic research:** the work repeatedly distinguishes clinician-led care, institutional research, compliant cosmetics and general education, and correctly rejects “research use only” language paired with human-use promotion.
- **Control-system design:** the SKU gate, supplier/site/product qualification, CoA authentication, landed-cost model, inventory logic, quarantine/release, FEFO, recall, CAPA and KPI structures are materially stronger than typical pre-launch work.
- **Procurement intelligence:** exact-pack comparisons, supplier-role mapping, pricing sources, RFI/RFP logic, negotiation levers and diligence requirements can be converted into a sellable institutional service.
- **Customer and partner architecture:** the audits define credible metabolic, healthy-ageing, aesthetic, laboratory and institutional journeys, including ethical handoffs, independent prescribing and no-commission principles.
- **Trust-led content foundation:** evidence levels, named review, sources, update dates, uncertainty and narrow CTAs provide a credible platform for an Evidence Desk after route approval.
- **Prudent caveats:** the project already states that forecasts are scenarios, keyword estimates require validation, a CoA is not proof of clinical fitness, supplier information does not prove live stock, and blank templates are not operational readiness.

The biggest missing systems

Missing system	What must exist before scale	Board implication
Operating thesis and entity map	One primary route; contracting entity; accountable professional/operator; licence and product-status map; permitted buyer, CTA and revenue mechanic	No public acquisition or inventory approval before sign-off
Offer and revenue architecture	Model-specific buyer, deliverable, price, billing cadence, cancellation terms, service capacity, partner economics and prohibited cross-sell	Replace generic enquiries with paid offer tests
Demand and learning system	Interview protocol, live RFQs, price tests, paid pilots, win/loss reasons, renewal signals and cohort measures	Treat all market size and ROAS as hypotheses
Unit economics and cash model	Fully loaded service cost, professional time, testing, logistics, tax, payment terms, working capital, expected loss and downside runway	Require contribution after analyst/clinical time, not gross revenue
Route-separated data architecture	Separate forms, CRM workspaces, consent text, analytics audiences, ledgers, staff permissions, retention and breach workflows	No shared “peptide” database or cross-lane retargeting
Executable quality and regulatory system	Named approvers, controlled SOPs, evidence repository, electronic approvals, expiry alerts, stop-publish/stop-ship actions, recall and restore drills	Blank workbook fields remain blockers
Partner contracting and independence	Fixed-fee service agreements, RACI, SLAs, conflicts, data roles, adverse-event routing, IP, termination and explicit no-referral economics	No commissions, prescription-linked or volume-linked payments
Local ecosystem and service blueprint	Verified George/Garden Route provider map, capacity, handoff codes, response SLAs, service recovery and affordability workflow	Build a local operating network before consumer demand generation
Data-rights and IP system	Permissioned aggregate-use rights, de-identification rules, sequence confidentiality, TTO/NIPMO screen and George background-IP policy	Data moat must be contractually earned; client IP cannot be presumed
Reputation and evidence publishing	Reviewer charter, corrections log, crisis response, source surveillance, claims expiry and aggregate quality metrics	Publish proof only after controls generate real evidence

Ranked opportunity portfolio

Scoring note. Scores are **strategic judgments, not measured forecasts**. Each idea is rated from 1 (low) to 5 (high). For **cost** and **compliance risk**, 1 is favourable/low and 5 is unfavourable/high. Ranking applies the following judgment weights: impact 25%; low cost 15%; speed 15%; compliance feasibility 20%; defensibility 15%; and George/Garden Route fit 10%. “Def.” and “Fit” are shown because they materially change the order. Tier 1 means **do now or initiate now**; Tier 2 means **build after proof**; Tier 3 means **park pending a specific trigger**. A star marks a particularly durable moat rather than a campaign.

Rank	Category	Idea	Why it matters	Minimum viable proof	Impact	Cost	Speed	Risk	Def.	Fit	Score	Decision tier
1	Governance infrastructure	★ Route firewall and machine-readable claims ledger	Removes the root ambiguity and turns compliance into reusable infrastructure	Select one primary route; map entities and licences; tag 25 claims, 10 pages and 10 messages; separate forms/CRM/consent; run leakage red-team	5	2	4	1	5	5	4.70	Tier 1 — Gate
2	B2B intelligence	★ Regulated-product procurement intelligence subscription	Monetises the strongest completed asset with no inventory or patient data; verification history compounds	Five design partners; dated exact-pack/status dashboard; monthly change log; three paid quarterly pilots and one renewal signal	5	2	4	1	5	4	4.60	Tier 1 — Lead offer
3	Local ecosystem	★ Garden Route Access Ledger and total-cost navigation	Creates a hard-to-copy map of capacity, price, benefit, stock and referral friction	Serve five practices for 12 weeks; 20 consented cost/benefit journeys; weekly verified update; reason-coded attrition	5	2	5	2	5	5	4.65	Tier 1 — Co-design now

Rank	Category	Idea	Why it matters	Minimum viable proof	Impact	Cost	Speed	Risk	Def.	Fit	Score	Decision tier
4	Institutional quality	★ Virtual Peptide Quality Office	Recurring research-lab revenue and embedded lot continuity without manufacturing or broad inventory	Two paying labs; five controlled templates; up to three lot reviews each; documented defects, time, contribution and renewal intent	5	2	5	2	5	3	4.45	Tier 1 — Paid pilot
5	Clinical network	★ Garden Route Clinical-Pharmacy Quality Compact	Converts the hub-and-spoke thesis into accountable local workflow and recurring support fees	Verify 5–7 organisations; appoint clinician and pharmacy leads; sign RACI; tabletop routine and incident journeys; 90-day charter	5	2	4	2	5	5	4.50	Tier 1 — Design, no traffic
6	Operating system	★ Fail-closed low-code control tower	Makes blank gates executable and produces an audit trail without enterprise software	Six linked records; five approval workflows; least privilege; backup/rest ore test; simulated stop-ship and stop-publish	5	2	4	2	5	5	4.30	Tier 1 — Minimum build

Rank	Category	Idea	Why it matters	Minimum viable proof	Impact	Cost	Speed	Risk	Def.	Fit	Score	Decision tier
7	B2B diligence	Fixed-fee supplier diligence and tender-readiness desk	Fast path from checklist to priced deliverable; complements subscription revenue	One molecule/presentation, three bidders, 10-day SLA, evidence index, exceptions register and annual-monitoring quote	4	2	5	2	4	3	4.05	Tier 1 — Attach offer
8	Regulatory intelligence	★ Watch tower with human-approved kill switches	Links recalls and evidence expiry to inventory and channels; creates change-history data	Source register and review cadence; match status/expiry to records; simulate recall; measure detection-to-block time	5	2	4	1	5	4	4.60	Tier 2 — After master data
9	Research data	★ Permissioned South African Peptide Project Index	Converts each legitimate RFQ and lot into better quoting, supplier scoring and future benchmarks	Agree data rights; define 25 fields; capture 10 RFQs and five accepted lots; keep first dashboard internal	5	2	3	3	5	3	3.80	Tier 2 — Spine for VQO

Rank	Category	Idea	Why it matters	Minimum viable proof	Impact	Cost	Speed	Risk	Def.	Fit	Score	Decision tier
10	Trust and reputation	★ George Trust Ledger	Makes corrections, evidence expiry and aggregate process performance publicly accountable	Publish methodology, boundaries, reviewer register, correction log and five non-promotional process KPIs after evidence exists	4	1	5	1	5	5	4.60	Tier 2 — Publish after proof
11	Customer continuity	90-day Care Continuity Sprint	Tests real drop-off, safety and coordination in one clinician-led pathway	25–30 consented participants; day 0/7/21/42/84 cadence; reason-coded exits; no prescription or outcome promise	5	2	4	3	4	5	4.00	Tier 2 — Clinical gates first
12	Organic evidence	Evidence Desk plus Question-to-Answer flywheel	Builds a source-linked question dataset and reusable content without promoting products	One reviewed briefing, three source cards, regulatory explainer, corrections log and weekly de-identified FAQ process	4	2	5	1	4	4	4.35	Tier 2 — After route approval

Rank	Category	Idea	Why it matters	Minimum viable proof	Impact	Cost	Speed	Risk	Def.	Fit	Score	Decision tier
13	Research supply	Institutional procurement evidence room	Document-first sourcing support can earn recurring fees before inventory	Three verified institutional buyers; eligibility and intended-use gates; due-diligence room; no individual accounts	4	2	3	3	4	3	3.35	Tier 2 — After classification
14	Pharmacy quality	Pharmacy batch passport and recall-drill service	Makes traceability tangible and creates recurring readiness work	One pharmacy and lawful product route; versioned lot record; synthetic-patient mock recall; record completeness test	4	2	4	2	5	4	4.10	Tier 2 — Partner dependent
15	Local provider data	Neutral Care-Capacity Atlas and closed-loop referral compact	Verified scope, language, access and referral closure can become a local public utility	Verify 20–30 providers; recruit 8–12; factual directory; four status codes; two-day acknowledgment SLA; no ranking fees	4	1	4	2	4	5	4.10	Tier 2 — Build with compact

Rank	Category	Idea	Why it matters	Minimum viable proof	Impact	Cost	Speed	Risk	Def.	Fit	Score	Decision tier
16	Research services	George handling and stability evidence network	Local handling evidence can convert a logistics disadvantage into technical know-how	One qualified analytical partner; one low-risk research peptide; baseline plus three-condition micro-study	4	3	3	2	5	4	3.65	Tier 2 — After VQO demand
17	IP services	TTO-ready peptide invention evidence packs	Deepens university relationships while preserving provenance under South Africa's public-R&D IP regime	Co-design with one TTO and patent attorney; pilot a non-confidential retrospective dossier; no legal opinions	4	2	4	3	4	2	3.45	Tier 2 — Specialist add-on
18	Logistics evidence	George qualified-lane lab and reusable pack-outs	Produces local lane-performance data and continuity options	Challenge one lawful SKU and Cape Town—George lane; calibrated loggers; custody scans; internal lane card and backup	4	3	3	2	5	5	3.75	Tier 3 — Demand trigger

Rank	Category	Idea	Why it matters	Minimum viable proof	Impact	Cost	Speed	Risk	Def.	Fit	Score	Decision tier
19	Education/community	Health Literacy Salons and professional Evidence Academy	Low-cost trust and question generation, but weak moat unless tied to the evidence system	One no-pitch hybrid session and one paid professional webinar; independent review, source list and separate consent	3	2	4	2	3	5	3.50	Tier 3 — Support channel
20	Cosmetics	Separate cosmetic proof sprint and ingredient observatory	Can test a lower-complexity lane, but distracts from higher-fit B2B assets	Regulatory review of one formulation; 25-product neutral index; 30-person usability pilot; no therapeutic claims	3	3	3	2	3	3	3.00	Tier 3 — Park

The numerical order is a guide, not false precision. Dependencies alter sequencing: for example, the Trust Ledger scores highly but must not be published until real controls produce real evidence; the clinical compact can begin partner co-design now but cannot receive patient traffic until professional, privacy, product and incident controls are operational.

Top five do-now initiatives

Priority	Initiative	Accountable owner role	First deliverable	Success test
1	Route decision, entity map and route firewall	Board sponsor with Regulatory/Legal Lead	Board-approved route thesis; RACI; licence/status memo; separate brand, CRM, consent, claims and CTA map	One primary revenue lane approved; every unresolved activity marked blocked; red-team finds no cross-lane data or CTA leakage

Priority	Initiative	Accountable owner role	First deliverable	Success test
2	Procurement intelligence subscription with diligence attach	B2B Product and Procurement Lead	Founding offer sheet, dated data dictionary, source/refresh SOP, conflict policy and five buyer interviews	Three paid institutional pilots, at least one credible renewal signal and positive contribution after analyst/reviewer time
3	Virtual Peptide Quality Office	Institutional Services Lead with Peptide Quality Reviewer	Five controlled review templates, limited-scope terms, secure workspace and two laboratory pilot proposals	Two paying labs, five completed lot/RFQ reviews, documented defects caught, agreed data rights and renewal intent
4	Garden Route Quality Compact plus Access Ledger co-design	Partnerships Lead with independent Clinical Lead	Verified shortlist; one clinician, one licensed pharmacy and one laboratory commitment; draft RACI and neutral ledger	Signed 90-day charter, routine and incident tabletop passed, 20 cost/access journeys ready to measure; no referral economics
5	Minimum fail-closed evidence/control system	Quality Systems and Data Protection Lead	Permissioned records for route, claim, counterparty, evidence, consent and expiry; backup and manual fallback	Zero bypassed blocks in simulation; stop-publish/stop-ship works; restore test passes; complete timestamped evidence for one offer

Five initiatives to build after proof

- 6 **Regulatory watchtower with kill switches.** Add automated monitoring only after canonical master data and named human reviewers exist. The output is a review queue, not an automated regulatory decision.
- 7 **George Trust Ledger.** Publish corrections, review dates, complaints performance and aggregate quality metrics only when each metric has an owner, method and privacy threshold.
- 8 **Care Continuity Sprint.** Start only after the compact has a registered clinical lead, licensed pharmacy handoff, POPIA allocation, escalation SOP, full-price disclosure and adequate capacity.
- 9 **Permissioned Project Index and external benchmarks.** Use the index internally from day one of the Quality Office; sell or publish aggregates only after contracts, minimum-cell rules and sufficient observations make re-identification unlikely.
- 10 **Batch passport, recall drill and qualified-lane services.** Add these when a partner pharmacy or institutional client creates genuine recurring volume; do not build proprietary cold infrastructure first.

Ideas to park or avoid

Park until explicit evidence triggers

- **Cosmetic SKU or routine club:** reconsider only after the core B2B offers achieve renewal and a completely separate cosmetic entity/brand/CRM, product information file and claims review are affordable.
- **Pooled procurement of analytical standards:** require three institutional purchase orders, buyer-specific specifications, classification/customs review and no speculative stock.
- **Custom software:** manual and low-code pilots must first prove workflow stability, user retention, contribution and data rights.
- **Owned cold room, warehouse or national stock position:** require signed demand, qualified lanes, expiry protection, partner-capacity failure and a downside cash case.
- **Broad community membership:** require a clinical owner, moderation and escalation SOP, privacy architecture and evidence that continuity—not product consumption—drives value.

Avoid as structurally inconsistent with the strategy

- Direct consumer sale, promotion or facilitation of unregistered, compounded, grey-market or research-use-only peptides for human use. SAHPRA specifically warns against unregistered peptides sold online and through informal channels.[1]
- A mixed “peptide” storefront, shared WhatsApp number, shared CRM audience or hidden handoff among education, clinical, research and cosmetic lanes.
- Referral commissions, per-lead/per-prescription fees, volume rebates, paid preferential placement or sponsor influence over clinical or technical judgment.
- Public dosing, cycles, transformations, before/after imagery, guaranteed outcomes, unapproved comparisons or “medical/pharmaceutical grade” wording unsupported for the exact product and channel.
- Using a CoA as sufficient proof of identity, sterility, lawful marketability, clinical safety or lane suitability.
- Buying inventory or accepting deposits before exact-product status, responsible entities, supplier/site/lane qualification, landed economics and recall ownership are documented. SAHPRA provides an exact-product registration database, reinforcing the need to verify product and status rather than infer legality from an ingredient name.[2]
- Cold email, scraped WhatsApp lists, purchased health-interest audiences or cross-lane retargeting. POPIA’s direct-marketing framework requires a lawful basis, proof of consent where relied upon, sender identification, objection handling and suppression.[4]
- Treating the R22,000 six-month paid-media scenario, R36,000 90-day launch budget, supplier price comparisons or SEO volume estimates as validated demand, margin or cash flow.

30-day action sprint

Days	Workstream	Required actions	Output and gate
1–5	Board thesis and governance	Appoint Board Sponsor, Regulatory/Legal Lead, Quality Lead, Data Protection Lead and commercial owners. Choose the primary route and one secondary discovery lane. Define “not now.”	Signed one-page operating thesis, entity/licence map, RACI and blocked-activity list
3–8	Offer and economics	Write offer cards for procurement intelligence, diligence and Quality Office: buyer, problem, deliverable, SLA, fixed price, billing, capacity, exclusions and cancellation. Build a 12-month driver-based P&L/cash model with downside case.	Board-approved pilot prices; fully loaded contribution hurdle; maximum cash-at-risk
5–12	Route firewall	Separate domains/subdomains as needed, CRM workspaces, forms, consent, analytics, claims and permissions. Create a 25-row claims ledger and keyword exclusion list.	Red-team leakage test passed; no unresolved product claim published
6–15	Evidence system	Configure low-code records for route, claim, source, counterparty, product/service, evidence, expiry and consent. Add named approvals, change log, backup and suppression list.	One offer traverses end-to-end approvals; restore and stop-publish simulations pass
8–20	Primary validation	Conduct 10 procurement-buyer interviews, 10 laboratory interviews/RFQ discussions and 10 Garden Route partner interviews. Ask for paid action, not stated interest. Record objections and willingness to renew.	At least five written pilot proposals; interview evidence repository; win/loss taxonomy
12–24	Partner and legal pack	Draft fixed-fee MSA/SOW, conflicts disclosure, no-referral clause, data roles, IP/data rights, limitations, SLA, adverse-event/incident routing and termination. Verify registrations/licences.	Execution-ready contracts; verified candidate file; unresolved exceptions logged
16–28	Pilot conversion	Sell procurement pilots and two Quality Office pilots. Obtain compact design commitments from clinician, pharmacy and laboratory. Do not buy stock or acquire medicine-seeking consumers.	Target: three paid intelligence pilots, two paid Quality Office pilots, three compact anchor commitments
24–30	Decision review	Review paid demand, contribution, owner capacity, compliance exceptions and evidence-system performance. Select 60–90-day pilots; stop weak offers.	Written continue/revise/stop decisions with cash caps, metrics and next review date

Key dependencies and non-negotiable gates

- 11 South African regulatory and legal classification.** Obtain route-, activity- and product-specific advice; no generic “peptide legality” opinion is sufficient.

- 12 **Named accountable professionals.** Clinical activity requires independent professional governance; pharmaceutical handling and dispensing remain with appropriately licensed premises and professionals; research quality work requires competent technical review.
- 13 **Current exact-product and counterparty evidence.** Verify registration/status, approved information, licences, supplier/site roles and any restrictions at the decision point.[2]
- 14 **POPIA and PAIA operating pack.** Confirm Information Officer responsibilities, processing inventory, operator agreements, minimal fields, role-based access, retention/deletion, incident response, consent evidence and suppression.[4]
- 15 **Professional independence and fair-market-value contracts.** Clinical choice, prescribing, dispensing and awards must remain with authorised decision-makers; George fees must pay for real services rather than referrals or volume.
- 16 **Controlled evidence and claims.** Every external claim requires owner, route, exact object, evidence grade, permitted audience/channel, approval, expiry and correction history. No approval means no publication.[3]
- 17 **Data and IP rights.** Institutional contracts must allocate confidentiality, sequence ownership, client foreground IP, George background methods, aggregate-use rights and permitted retention. NIPMO oversees IP arising from publicly financed R&D under South Africa's IPR Act, so university work needs an early TTO/public-funding screen rather than a generic vendor IP clause.[6]
- 18 **Quality and resilience.** Before any physical-product activity, qualify supplier, site, product/grade, laboratory and lane; define quarantine/release, temperature evidence, complaints, returns, recall, destruction, backup providers and downtime procedures. SAHPRA wholesaling guidance emphasises quality systems, CAPA, documented returns and reconciled recall records.[5]
- 19 **Insurance and liability allocation.** Secure appropriate professional indemnity, cyber and product/logistics cover for each actual service; define failed-lot, misinformation, privacy, storage and incident liability.
- 20 **Board-enforced capital discipline.** No inventory, warehouse, custom app or scaled media until a paid manual pilot demonstrates demand, positive contribution after full labour/quality cost, renewal behaviour and bounded downside cash exposure.

Decision requested

The board should approve a **90-day asset-light proof phase** with a strict split between: (a) a primary institutional assurance business—procurement intelligence, fixed-fee diligence and a Virtual Peptide Quality Office; and (b) a separately governed Garden Route clinical-network design track that has no patient acquisition until all launch gates pass. It should decline broad consumer marketing, speculative stock and further brand exploration during this phase.

At day 90, continue only if the venture has: **at least three paying intelligence customers, two paying Quality Office customers, one documented renewal or expansion signal in each lead offer, positive contribution after professional/analyst time, a signed Garden Route compact with accountable clinician and pharmacy leads, zero bypassed route controls, tested incident/restore procedures and an evidence-based 12-month cash plan.**

If these tests fail, the board should narrow or stop the relevant lane rather than compensate with more traffic.

References

[1]: [South African Health Products Regulatory Authority, “Peptide Products – Public Information”](#) (accessed 21 August 2026).

[2]: [South African Health Products Regulatory Authority, “Registered Health Products Database”](#) (accessed 21 August 2026).

[3]: [South African Health Products Regulatory Authority, *Guidelines for Advertisement of Medicines and Health Products, SAHPGL-INSP-RC-07, Version 2*](#) (June 2022).

[4]: [Information Regulator South Africa, *Guidance Note on Direct Marketing in Terms of POPIA*](#) (accessed 21 August 2026).

[5]: [South African Health Products Regulatory Authority, *Guideline on South African Good Wholesaling Practice for Wholesalers, SAHPGL-INSP-03, Version 5*](#) (18 September 2022).

[6]: [National Intellectual Property Management Office, “About NIPMO”](#) (accessed 21 August 2026).